Pivotum

PIVOTUM CAREER VALUE GUIDE · FALL 2026

Is Veterinary Medicine Safe From AI?

The already-in-it guide — the full scores by track, the six factors behind them, and what to do from where you already are.

2.7

AI EXPOSURE · SMALL ANIMAL GENERAL PRACTICE

where 10 is most at risk

For anyone already in it — in the degree, or job-hunting.

HIGHLIGHTING IS COLOR-CODED

THE FINDING EXPOSURE PROTECTION METHOD & HONESTY

Before you read this

The choice is already made — you're in veterinary training, or out of it and into the job hunt. So this guide skips the "should you study veterinary medicine" question and goes straight to the one that's actually live: given where you already are, what do you do about it?

It's written to you directly — whether *you* are the one in it, or a parent reading on their behalf. Where it says "you," a parent can simply read "them"; the moves are the same either way.

What this is. A facts document, not a pep talk. It sets out what's known about where veterinary medicine sits in an AI economy, where the evidence is contested, and where we might be wrong — and then what to actually do from here.

The good news, up front. On AI exposure, you have already chosen well — veterinary is one of the safest careers in this entire index, and the clinical core you're most likely headed for is about as protected as work gets. That part is genuinely reassuring, and nothing on the horizon changes it much. The moves still in front of you matter less for automation than for the two things that actually shape a veterinary career: the debt and the emotional load. Both are still substantially in your hands from here, and the second half of this guide is about them.

And one thing specific to veterinary medicine. The profession openly discusses mental health and wellbeing in a way most don't, because it has genuine occupational stressors that are unusual and worth understanding properly. Euthanasia, cost-constrained care and client conflict are real parts of the work, and the profession has built substantial support around them precisely because they matter. Taking that support seriously — as a habit rather than a last resort — is one of the more important moves this guide points at.

The short answer

Yes on AI exposure, and this is one of the safest careers we score.

Small animal general practice scores **2.7** out of 10. Large animal and emergency work scores **2.5**. Veterinary nursing scores **3.0**.

The protection is unusually complete: physical work, licensed practice, deep owner trust, and a patient who cannot tell you what is wrong.

The reasons to think hard about veterinary medicine are entirely different ones — competitive entry, substantial training debt, and an emotional load the profession takes seriously. Where you land inside the profession, and how you handle those realities, is decided mostly by moves still in front of you.

The scores

TRACK	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Emergency & critical care	2.0	2.2	**2.4**	+0.4	Very low
Large animal / mixed practice	2.0	2.2	**2.5**	+0.5	Very low
Veterinary specialist / surgery	2.0	2.2	**2.5**	+0.5	Very low
Small animal general practice	2.2	2.5	**2.7**	+0.5	Very low
Veterinary nurse / technician	2.5	2.7	**3.0**	+0.5	Low
Lab / diagnostic veterinary roles	4.6	5.0	**5.4**	+0.8	Moderate

Scores run 1–10, where 10 is most at risk. For context: the median profession in this edition sits around 5.5, surgery scores 2.1, physical therapy 2.5, bedside nursing 2.8, and entry-level software development 8.1.

Read the table this way, and note the last row. Veterinary work moved behind a microscope or a screen scores double the clinical tracks — the same pattern that appears in every profession we measure. Your job, from here, is to stay on the clinical side of that line — not to hope the table moves.

Why veterinary scores so low — the six factors

How much of this job can AI already do?	5.2
How hard will it be to land that first job?	2.5
Does it have to be done in person, with your hands?	9.0
Does someone need a human they can trust and hold responsible?	9.0
Does the law require a licensed human?	9.0
How often does the job hit genuinely new, high-stakes situations?	9.0

EXPOSURE RAISES THE SCORE   PROTECTION LOWERS IT

Here's how small animal general practice answers them.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

The administration, the imaging support, and very little else.

Clinical records and history notes. Imaging support and first-pass reads. Differential diagnosis suggestions. Practice admin, reminders and billing. Protocol and dosage lookup. Client communication drafting.

Veterinary practices are buried in administration, and the automation landing there is widely welcomed rather than feared.

What resists: physical examination of an animal that resists it, surgery, handling a frightened or aggressive patient, telling an owner their dog is dying, and judgment when the patient cannot describe anything. Those are the things to be building toward.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Strong protection. Licensure requires supervised clinical training that cannot be automated or offshored, and veterinary school places are capped — which makes entry fiercely competitive but employment afterwards secure.

The bottleneck here is getting *in*, not getting hired after. Once qualified, employment is secure in a way almost nothing else in this index can claim — the hard part sits before the degree, not after it. If you're through admission, the scarce part is largely behind you. The work now isn't landing a job at all costs — it's landing the *right* setting, and treating relief work and employer loan repayment as deliberate strategies for the debt rather than afterthoughts. Those are concrete moves, and the career-hunting checklist further down lays them out.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

Almost entirely. Physical examination, procedures, surgery, restraint. The section on the patient who can't talk explains why this combines with something else to make veterinary unusually protected.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Yes. Owners are making decisions about an animal they love, often under financial pressure, and a licensed veterinarian is professionally answerable for the outcome.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Comprehensively. Veterinary practice, prescribing and surgery are all legally reserved to licensed practitioners.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

We rate this 9.0, and veterinary earns it differently from every other career in this index. The section that follows explains why.

The patient can't talk — and why that matters more than it sounds

AI is strong on patterns and weak on genuine novelty. So this factor measures how often the work presents something that doesn't match anything in the training data, where being wrong is costly.

Most careers earn a high rating here through complexity — an engineer's one-off site, a litigator's unprecedented case, an ICU nurse's crashing patient.

Veterinary earns it through a structural absence: there is no history.

Human medicine begins with a patient describing symptoms — where it hurts, when it started, what makes it worse, what they've taken. That narrative is the single richest input in clinical reasoning, and it is *text* — which is precisely what language models handle best.

A veterinary patient supplies none of it. No complaint, no timeline, no symptom description. Instead there is an animal that may be hiding pain by instinct, an owner's second-hand and often inaccurate account, and whatever the vet can determine by touching an animal that does not want to be touched.

Then multiply by species. A veterinarian in mixed practice may see a dog, a horse and a rabbit in one morning — three entirely different physiologies, drug tolerances and normal ranges.

That combination is unusually hostile to automation. Diagnostic support systems perform well when fed structured findings. Veterinary work is largely about *generating* those findings from a non-verbal, uncooperative, physically present patient — which no system can do.

The general lesson, and it transfers well beyond this profession: ask where a job's information comes from. Careers where the inputs arrive as text or structured data — analysts, paralegals, desk journalists — are exposed, because that is the form AI reads best. Careers where a human must generate the information from the physical world first are protected. Veterinary is the purest example of it in this index.

The financial picture — and it has been moving

This is the part that deserves the most attention, and the direction of travel matters as much as the level.

The AVMA's own data — the profession's annual Graduating Senior Survey — tells a two-part story.

It improved. The average debt-to-income ratio for new graduates entering full-time employment fell from 1.70 in 2021 to 1.44 in 2022, and stood at **1.4 for 2024 graduates**. That approaches the figure generally considered serviceable without serious financial stress.

Then it started climbing again. Average DVM debt across all new graduates — including those with none — rose to $174,484 in 2025, up from $153,972 in 2022. For those carrying debt, the average reached $212,499, up from $186,788.

The AVMA's own framing is blunt: a few years ago it looked as though the profession might be turning a corner, and more recent data show the picture becoming complicated again.

And the average conceals a wide spread. More than a quarter of new graduates in one recent cohort carried a debt-to-income ratio of 2.0 or higher — roughly double what professionals consider healthy, which is closer to 1.

Against that, median veterinary pay sits around $125,510 according to BLS, with real geographic variation: Massachusetts, California and Hawaii average above $155,000, while rural states can run $20,000–$30,000 lower. Emergency and specialty practice pay more than general practice.

Two practical things worth knowing

Relief work changes the arithmetic. Relief veterinarians work per-diem shifts across multiple practices rather than as full-time employees. General practice relief rates run roughly $55–$125 an hour, with experienced practitioners typically $70–$95 and day rates of $600–$1,000. A relief vet working four days a week can earn $120,000–$160,000. It's a genuine strategy for accelerating debt repayment, and few applicants know it exists.

And the AVMA has explicitly told applicants they do not need enormous quantities of unpaid veterinary experience to be admitted. That guidance exists precisely because the expectation was pushing people into years of unpaid work before an already expensive degree. If you're already in, the relevant version is this: don't let the same instinct push you into unpaid or underpaid work now that you don't actually need.

Employer loan repayment assistance is also emerging — some large practice groups now contribute several hundred dollars a month toward veterinary student debt, which materially shortens a payoff timeline. It is a legitimate thing to ask about at interview.

What the trend shows

Small animal practice: 2.2 → 2.5 → 2.7. Up 0.5 in three years, and all of it administrative.

Lab and diagnostic roles: 4.6 → 5.0 → 5.4. Up 0.8.

The gap widened from 2.4 points to 2.7. Same pattern as everywhere: the further the work moves from the animal, the higher the exposure climbs.

How durable is the protection?

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Judgment under novelty	**Among the highest we score**	**Most durable here**	The absence of patient history is structural, not incidental.
Regulatory / licensing	Strong	**Most durable**	Practice, prescribing and surgery are legally reserved.
Human trust / accountability	Strong	**Durable**	Owners making hard decisions want a person answerable.
Physical / embodied	Very strong	**Most fragile long-term**	The protection physical AI targets — though animals are among the hardest subjects for robotics.

The honest read. Veterinary medicine has all four protections working together, which is rare. The physical one carries the usual long-horizon caveat, but restraining and examining an uncooperative animal is among the harder problems in robotics, not among the easier ones.

How the role will actually change

WAS THE JOB	IS BECOMING THE JOB
Writing up the consultation	Reviewing an ambient draft
Recalling drug doses across species	Instant lookup, with judgment about whether it applies
Reading the radiograph unaided	First-pass read arrives; the vet adjudicates
Practice admin consuming evenings	Substantially automated
Examining the animal	**Unchanged**
Telling an owner their animal is dying	**Unchanged**

The pattern is unusually clean here. In veterinary medicine the automatable layer and the clinical layer are cleanly separable, and what is automating is the paperwork — which is a leading complaint in the profession.

Human strengths that will matter most

1. **Physical examination of a non-verbal patient** — generating the clinical picture from scratch. The least automatable thing in this index. 2. **Handling animals** — restraint, reading fear and aggression, working safely with a patient that doesn't understand you're helping. 3. **Difficult conversations with owners** — euthanasia, cost, prognosis. Consistently named as the hardest and most human part of the work. 4. **Judgment across species** — knowing what normal looks like in three different physiologies in one morning. 5. **Accountability** — carrying professional responsibility for an animal's life.

And underneath all five, two traits the score can't grade: curiosity and adaptability. The specific tools — the ambient scribe, the imaging first-pass read, the diagnostic-support engine — will turn over many times across a veterinary career. The clinicians who compound are the ones who stay genuinely curious about the animal in front of them and re-tool fastest, rather than defending the one way of working they trained on. In a profession this well protected, that flexibility is less about survival than about staying good at the work as its tools change around you — and it's increasingly what employers hire *for*: the trajectory, not the current toolkit.

Other things to consider about this job

What's genuinely good about it

The work is described in vocational terms more consistently than almost any career we score. People don't drift into veterinary medicine; they choose it and mostly keep choosing it.

- **Extraordinary variety** — species, presentations, settings, and no two days alike
- **High autonomy**, and practice ownership as a realistic destination
- **Surgical and medical work combined** in a way human medicine separates
- **Immediate, visible results** in much of the work
- **Genuine relationships with clients** built over an animal's lifetime

And the financial picture is better than the debt figure alone suggests: BLS median around $125,510, emergency and specialty practice paying above that, and relief work offering a route to $120,000–$160,000 with flexibility.

What's genuinely hard about it

Entry is fiercely competitive. Veterinary school places are capped and admission is demanding — in some markets harder than medical school. If you're through it, the hardest gate is behind you.

The debt is substantial and rising again, covered in the financial picture above.

And the emotional load is real, unusual, and worth understanding properly.

Three things make it distinctive:

- **Euthanasia is routine.** Few careers involve ending life regularly, and it does not become trivial.
- **The financial constraint on care.** A veterinarian frequently knows exactly what would help an animal and cannot provide it because the owner can't afford it. That situation has no real equivalent in human medicine, and practitioners consistently name it among the hardest parts of the job.
- **Client conflict**, sometimes severe, often at moments of grief or financial stress.

The profession discusses this openly, which is to its credit and unusual. The AVMA and equivalent bodies internationally run substantial wellbeing programs, peer support networks and confidential assistance services, and veterinary schools now build this into training rather than leaving graduates to discover it.

Better to know this going in than to find out at twenty-six. It is not a reason to leave the profession. It is a reason to work in it with open eyes, to take the support seriously, and to choose a practice culture carefully.

Who this work suits — and who it doesn't

Veterinary medicine tends to suit people who:

- **Are interested in animals *and* comfortable with people.** The most common misapprehension is that this is a career for people who prefer animals to humans. Much of the job is managing owners.
- **Are physically confident with animals** — including frightened, aggressive and large ones
- **Can hold competing pressures** — clinical, financial and emotional, in the same consultation
- **Are practical problem-solvers** rather than pure academics
- **Can carry difficult things** without being flattened by them

It's harder for people who:

- **Struggle with conflict**, particularly around money
- Find **grief** hard to be around repeatedly
- Want **predictable hours** — emergency and on-call work is common
- Came to it primarily because they **love animals**. That's the starting point, not the job.

If some of that second list sounds like you, it isn't a reason to abandon the path — it's a reason to steer toward the settings that fit and to lean on the support that exists, which is what the rest of this does.

What else is moving this market

Demand is stable and pet ownership is high, though the economics of veterinary care are under pressure from affordability at the client end.

Corporate consolidation of practices has changed the profession's structure considerably, with implications for autonomy, culture and ownership prospects that vary enormously by group.

And rural and large-animal practice faces persistent shortages, with government loan repayment programs available in high-need areas — worth investigating for anyone drawn that way.

The AI-native advantage — what it actually looks like

The situation: veterinary AI is arriving as administrative relief and diagnostic support rather than clinical substitution, which makes this one of the more straightforwardly positive pictures in the index. Being already in it, close to the work, is a real advantage here — you can build this fluency inside a live rotation, internship or first job, which is exactly where it counts.

WHAT AN AI-NATIVE VETERINARIAN LOOKS LIKE

1. Verify. Knowing when an ambient consultation note or a first-pass image read is wrong. A clinical skill, not a technical one — it depends on having examined the animal yourself.

2. Use diagnostic support without deferring to it. These tools are genuinely useful and genuinely confident when wrong.

3. Reclaim the administrative time deliberately. Practice admin is the layer automating. Whether that time returns to patients or is absorbed by higher caseloads is partly a personal choice and partly a practice-culture question — worth asking about at interview.

4. Handle owner AI use. Clients arrive having consulted symptom checkers and forums. Being able to work with that without dismissing it is a genuinely new client-communication skill.

5. Understand the limits. Diagnostic systems trained predominantly on one species or population behave unpredictably outside it, which matters more in veterinary than almost anywhere.

All five are available to you *now* — inside a current rotation or first job, not after some future qualification. Turning them into concrete habits is the rest of this guide.

Where a veterinary degree can take you later

Broad: clinical practice across species and settings, specialization, practice ownership, research and academia, public health and epidemiology, government and regulatory work, industry (pharmaceutical and nutrition), conservation and wildlife, and shelter medicine.

The pattern in this index holds: the further from the animal, the higher the exposure. Lab and diagnostic roles score 5.4 against clinical practice at 2.5. That pattern is the compass for every choice below.

Repositioning from here

The program-selection stage is behind you. The useful questions now are different — and for veterinary the good news is unusually solid: the clinical core you're most likely headed for is already among the safest work in this entire index. The table runs from clinical practice at 2.4–2.7 up to lab and diagnostic roles at 5.4, and the honest truth is that where you land inside it matters less for AI exposure than for the two things that actually shape a veterinary career: the debt and the setting. Both are still in your hands from here.

If you're still in the degree or training

Three levers, in order of leverage.

1. Setting is the whole game — and it means staying close to the animal. The clinical tracks all cluster at the safe end: emergency and critical care (2.4), large animal and mixed practice (2.5), specialist and surgery (2.5), small animal general practice (2.7). The one exposed lane is lab and diagnostic work (5.4) — the same "behind a microscope or a screen" pattern that appears in every profession we measure. Get exposure across species and settings now — small animal, large animal, emergency, shelter — because the setting shapes the career more than the qualification does, and breadth is far harder to add later.

2. The debt, while you can still shape it. The average for graduates carrying debt reached $212,499, and more than a quarter of a recent cohort carried a debt-to-income ratio of 2.0 or higher. You can't undo the tuition, but you can shape the payoff: learn what relief work and employer loan repayment actually do before you graduate, not after. Four days a week of relief can earn $120,000–$160,000; some practice groups now contribute toward student debt. Both are legitimate to plan around and to ask about at interview.

3. Wellbeing, treated as training rather than triage. Euthanasia, cost-constrained care and client conflict are real, unusual and named by practitioners among the hardest parts of the work. The profession has built genuine support around them. Building the habit of using it now — while it's part of the curriculum — is far easier than reaching for it in a crisis later.

If you've graduated and you're job-hunting

Here the setting on the offer matters more than the title, because the exposure in this profession is almost entirely about how close the work stays to the animal.

- **Clinical, hands-on practice is the protected core (2.4–2.7).** Small animal, mixed, emergency, specialty — this is most of the profession and where the protection lives. Aim here on purpose.
- **The lab and diagnostic lane (5.4) is the one exposed setting.** A role that's mostly screen-based diagnostic reading drifts toward it. Taking one isn't fatal — but take it as a deliberate choice with your clinical hours in view, not as the default.
- **Chase the setting, then the debt strategy.** Relief work ($120,000–$160,000 four days a week) and employer loan repayment are real levers; large-animal and rural practice carry government loan repayment programs in high-need areas. Ask about all of it directly.
- **Choose practice culture as carefully as specialty.** Whether reclaimed admin time returns to patients or is swallowed by higher caseloads, and whether wellbeing support is real or nominal, are culture questions — and culture is what you actually live inside.

The AI-native move, from where you stand

You can't be anywhere but where you are, but you can be the vet who *directs* these tools rather than defers to them. The five capabilities above — verify, use diagnostic support without deferring, reclaim admin time, handle owner AI use, understand the limits — are all available right now, inside a current rotation or first job. The most powerful for someone already in it is the first: be the clinician who knows when the ambient note or the first-pass read is wrong. That's a clinical skill, not a technical one — it depends on having examined the animal yourself, which is exactly the thing no system can do — and you can start building it this rotation, not after some future qualification.

The career-hunting checklist

The analysis tells you where the exposure is. This is what to do about it — the part that's actually in your hands. Most of what shapes a veterinary career is built here, in these habits and choices, rather than handed to you. The first list is for anyone still in the degree or training; the second is for the job hunt itself.

During the degree / training — build what can't be automated

- **Stay on the hands-on, animal-facing side of the work.** The clinical tracks — emergency and critical care (2.4), large animal and mixed practice (2.5), specialist and surgery (2.5), small animal general practice (2.7) — are the protected core. Lab and diagnostic roles (5.4) are the one lane that scores like an office job. Aim your exposure at the animal, not the screen.
- **Get exposure across species and settings.** Small animal, large animal, emergency, shelter. The setting shapes the career more than the qualification does, and breadth is the thing you can't easily add later.
- **Shape the debt now, not at graduation.** The average for graduates carrying it reached $212,499. Learn what relief work and employer loan repayment can do before you leave — and don't let the old instinct push you into unpaid work you don't actually need.
- **Take the wellbeing support seriously from day one.** Euthanasia, cost-constrained care and client conflict are real and unusual; the profession has built genuine support around them. Use it as a habit, not a last resort.
- **Get fluent with veterinary AI as a clinician, not a shortcut.** Learn to direct the ambient scribe and the first-pass image read, and to catch them when they're confidently wrong — verification depends on having examined the animal yourself.
- **Feed curiosity and adaptability on purpose.** The tools will turn over repeatedly; the clinician who stays genuinely curious about the animal and re-tools fastest is the one who compounds.
- **Keep the frame wide.** Properly vet the adjacent paths — large-animal and rural practice, specialty training, veterinary nursing (3.0) if you're rethinking the depth of the

commitment — before ruling them out. The order in which you enter them matters.

During the job hunt — aim, don't just apply

- **Target clinical, hands-on practice on purpose.** Small animal, mixed, emergency, specialty — the animal-facing work is where the protection lives, and it's most of the profession.
- **Read the setting, not just the offer.** A role that's mostly screen-based diagnostic reading drifts toward the lab and diagnostic lane (5.4). If you take one, take it deliberately, with your clinical hours in view — not as the default.
- **Don't grab a role just because it's on offer in an exposed lane.** A role being available doesn't make it safe. A practice or lab building roles around behind-the-screen diagnostic work rather than hands-on clinical care may simply be behind the curve — and that's exactly the kind of work that scores 5.4 in this profession. Availability can be a lagging indicator, not a green light.
- **Investigate relief work and employer loan repayment as real debt strategies.** Four days a week of relief can earn $120,000–$160,000; some practice groups now contribute toward student debt. Both are legitimate to ask about at interview.
- **Choose practice culture as carefully as specialty.** Whether reclaimed admin time returns to patients or is absorbed by higher caseloads is a culture question — ask it directly, and ask what wellbeing support actually looks like in practice.
- **Ask the employer where AI is heading in the role.** Whether they're building the job around ambient notes and diagnostic support, or waiting to be forced to, tells you if the seat grows or is quietly being hollowed out.

Go deeper — further reading

- **AVMA — Economic State of the Veterinary Profession and student debt research** — *American Veterinary Medical Association*. The profession's own annual data on debt, starting salaries and debt-to-income ratios. The primary source for the financial picture here, and the most reliable available.
- **Veterinarians — Occupational Outlook Handbook** — *US Bureau of Labor Statistics*. Median pay, projected growth and entry requirements. Read the **Veterinary Technologists and Technicians** page alongside it.

- **VIN Student Debt Center** — the Veterinary Information Network's resource on educational debt and repayment. Practical rather than promotional, and written by people inside the profession.

The counterargument — read this one:

> The strongest case against our 2.7 isn't that AI will replace veterinarians. It's that a very low AI score may lead someone to under-weight the two risks that actually end veterinary careers: the debt and the emotional load.
>
> On this reading, our framework tells you that veterinary medicine is among the safest careers available — which is true — while saying nothing about a debt figure that has started climbing again, more than a quarter of recent graduates carrying debt-to-income ratios above 2.0, or the occupational stressors the profession itself treats as a serious wellbeing issue.
>
> A career can be automation-proof and still be difficult to sustain.
>
> **Where we land:** this is fair, and it's why the financial and emotional sections are longer here than the AI analysis. Our score measures automation exposure only. For veterinary medicine specifically, the AI answer is genuinely reassuring and the honest answer is more complicated — and conflating the two would be the most damaging thing this document could do.

Bottom line

On AI exposure, veterinary medicine is about as protected as work gets — which is exactly why being already in it is a strong position. All four of our protections operate together: physical examination, comprehensive licensure, deep owner trust, and a patient who cannot describe a single symptom. The absence of a spoken history is a structural protection nothing else in this index has.

The real considerations are financial and emotional, and both deserve serious weight. Debt averaging $212,499 for graduates who carry it, rising again after several years of improvement, with more than a quarter of a recent cohort above a 2.0 debt-to-income ratio. And an emotional load — routine euthanasia, financial constraint on care, client conflict at moments of grief — that the profession has built substantial support around precisely because it is real.

So the useful advice isn't a verdict on the path — it's a direction. Aim your rotations and your job hunt at hands-on clinical practice, where the protection lives, rather than drifting toward the lab and diagnostic lane. Treat relief work and employer loan repayment as genuine debt strategies rather than afterthoughts. Choose practice culture as carefully as specialty. And take the wellbeing support seriously from the start rather than when it's needed.

And one honest thing, if the ambition that got you here has started to feel heavier than it did. The instinct was a good one, and the career is genuinely protected in a way very few are. The work now is to carry the numbers and the difficulties alongside the ambition — not instead of it.

Questions worth sitting with

Whether you're the one in it or a parent thinking it through, these are the conversations that move the needle now. Read "you" as whoever the decision is about.

Where you actually stand

1. Which setting are you heading toward — and does it keep you hands-on with the animal (the 2.4–2.7 clinical core) or point you toward screen-based lab and diagnostic work (5.4)? 2. Looking at the score table, which track are you currently pointed at, and is it the one you'd choose on purpose? 3. Have you actually looked at your own debt number against a realistic starting salary — and do you know where it sits relative to a 2.0 debt-to-income ratio?

The move from here

4. What's the single most protected setting you could realistically aim for from where you are now, and what would make you a credible candidate for it? 5. If you're still studying: how much cross-species and emergency exposure are you actually getting, and is there more you could add while it's still part of the training? 6. If you're job-hunting: have you investigated relief work and employer loan repayment as real strategies, or are you applying to whatever's open? Name the settings you're actually chasing.

The AI question, practically

7. Of the five capabilities — verify, use diagnostic support without deferring, reclaim admin time, handle owner AI use, understand the limits — which one could you visibly demonstrate within the next few months? 8. Can you point to a moment where you caught an ambient note or a first-pass image read that was wrong? If not, where's the easiest place to start building that skill?

Testing it against reality

9. **The most valuable single action:** find a vet three to five years qualified and ask directly about their loan payments, their setting, and whether they'd choose the same path. 10**. Euthanasia, cost-constrained care, client conflict.** Would you actually use the wellbeing support the profession has built, or feel you should cope alone? Decide before you need it, not after.

This is analysis and informed judgment about a fast-moving situation. For veterinary medicine specifically, we've flagged that the profession's most serious risks — the debt and the emotional demands — sit entirely outside what our six factors can measure, and that a low AI score should not be read as an easy career.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology. Scores measure exposure to what AI can already do — not how much any particular employer has deployed.

Technical scoring appendix — Veterinary Medicine

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

1. The formula

```
Risk = 0.35 × Automatability
     + 0.15 × EntryErosion
     + 0.15 × (10 − Physical)
     + 0.15 × (10 − Trust)
     + 0.10 × (10 − Regulatory)
     + 0.10 × (10 − Judgment)
```

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-eight careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human*?

RATING	ANCHOR
0–2	No license exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent*, *where being wrong is costly*?

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

EMERGENCY & CRITICAL CARE — 2.4 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.4	4.8	**5.3**	1.85
Entry-path erosion	15%	1.2	1.5	**1.8**	0.27
Physical / embodied	15%	9.5	9.5	**9.5**	0.07
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	9.2	9.2	**9.2**	0.08
				Total	**2.41 → 2.4**

LARGE ANIMAL / MIXED PRACTICE — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	**5.4**	1.89
Entry-path erosion	15%	1.2	1.5	**1.8**	0.27
Physical / embodied	15%	9.5	9.5	**9.5**	0.07
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	9.0	9.0	**9.0**	0.1
				Total	**2.49 → 2.5**

VETERINARY SPECIALIST / SURGERY — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	**5.4**	1.89
Entry-path erosion	15%	1.2	1.5	**1.8**	0.27
Physical / embodied	15%	9.5	9.5	**9.5**	0.07
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	9.0	9.0	**9.0**	0.1
				Total	**2.49 → 2.5**

SMALL ANIMAL GENERAL PRACTICE — 2.7 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.6	5.3	**5.8**	2.03
Entry-path erosion	15%	1.5	1.8	**2.0**	0.3
Physical / embodied	15%	9.2	9.2	**9.2**	0.12
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	9.0	9.0	**9.0**	0.1
				Total	**2.7 → 2.7**

VETERINARY NURSE / TECHNICIAN — 3.0 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.4	4.9	**5.6**	1.96
Entry-path erosion	15%	1.8	2.0	**2.2**	0.33
Physical / embodied	15%	9.2	9.2	**9.2**	0.12
Human trust / accountability	15%	8.5	8.5	**8.5**	0.22
Regulatory / licensing	10%	8.5	8.5	**8.5**	0.15
Judgment under novelty	10%	8.0	8.0	**8.0**	0.2
				Total	**2.98 → 3.0**

LAB / DIAGNOSTIC VETERINARY ROLES — 5.4 (MODERATE)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.5	5.4	**6.4**	2.24
Entry-path erosion	15%	3.5	4.0	**4.5**	0.67
Physical / embodied	15%	4.0	4.0	**4.0**	0.9
Human trust / accountability	15%	5.0	5.0	**5.0**	0.75
Regulatory / licensing	10%	6.0	6.0	**6.0**	0.4
Judgment under novelty	10%	5.5	5.5	**5.5**	0.45
				Total	**5.42 → 5.4**

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Emergency & critical care

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.4	1.2	0.28	**2.0**
2025	4.8	1.5	0.28	**2.2**
Fall 2026	5.3	1.8	0.28	**2.4**

Movement decomposition: automatability contributed **+0.31** and entry-path erosion **+0.09**, for a total of **+0.4** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labor market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

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7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.